National Cancer Control Programme (NCCP) 00798 · clinician draft

# Phesgo + vinorelbine

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## intravenous (IV) vinorelbine · Days 1 and 8

**Active medicines:** Pertuzumab / trastuzumab (Phesgo) + IV vinorelbine (HER2 treatment on Day 1 only, if still prescribed)

**Emetogenic risk:** Minimal · IV route

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

### Additional phase-specific support

- If HER2 therapy has stopped, the IV or oral vinorelbine plan may still apply, but remove the stopped antibodies from the prescription. Confirm the active components at every visit.
- Confirm Days 1 and 8 only, active vinorelbine route, current counts and the actual prescribed dose. IV and oral doses are not interchangeable.
- Assess constipation; agree prophylactic laxative or dietary support where appropriate. Severe pain, vomiting or no stool/wind requires urgent review.
- Antibody premedication is usually unnecessary unless indicated by previous hypersensitivity; consider paracetamol/antihistamine per local policy.
- No routine IV taxane steroid or carboplatin antiemetic course applies. An individual rescue plan may still be needed.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00798 v3 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-21 · SACTCheck v0.84.16 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

## Oral vinorelbine · Days 1 and 8

**Active medicines:** Pertuzumab / trastuzumab (Phesgo) + Oral vinorelbine (HER2 treatment on Day 1 only, if still prescribed)

**Emetogenic risk:** Oral moderate–high · National Cancer Control Programme (NCCP) oral classification

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron (one 5-HT3 option) | 8–16 mg total orally per day | On each prescribed oral vinorelbine treatment day; exact dose, timing and duration to be completed<br>Choose ONE 5-HT3 regimen. Review QT, hepatic dosing, constipation and interactions; granisetron is an alternative, not an add-on. |

### Additional phase-specific support

- If HER2 therapy has stopped, the intravenous (IV) or oral vinorelbine plan may still apply, but remove the stopped antibodies from the prescription. Confirm the active components at every visit.
- Confirm Days 1 and 8 only, active vinorelbine route, current counts and the actual prescribed dose. IV and oral doses are not interchangeable.
- Assess constipation; agree prophylactic laxative or dietary support where appropriate. Severe pain, vomiting or no stool/wind requires urgent review.
- Antibody premedication is usually unnecessary unless indicated by previous hypersensitivity; consider paracetamol/antihistamine per local policy.
- NCCP oral-agent guidance recommends a 5-HT3 antagonist: ondansetron 8–16 mg total daily OR granisetron 1–2 mg total daily. Confirm the intermittent Day 1/8 prescription; do not automatically give a daily 21-day course.
- Do not copy the minimal IV vinorelbine classification to the oral route. Do not automatically copy a high-risk IV NK1/steroid schedule.
- Swallow capsules whole with water and some food. No extra dose after vomiting; contact the team.
- Source distinction: NCCP regimen supportive section labels vinorelbine minimal without separating routes; NCCP2025 nausea/vomiting classification Table8 places oral vinorelbine in moderate–high and Table5 gives oral prophylaxis.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00798 v3 · p6 · NCCP antiemetics v8, Table 1 pp4–5

NCCP oral guidance v6: Tables5/8, pp8/13 · Sources checked 2026-10-21 · SACTCheck v0.84.16 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

## HER2-only continuation · if prescribed

**Active medicines:** Pertuzumab / trastuzumab (Phesgo)

**Emetogenic risk:** Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

### Additional phase-specific support

- Confirm which medicine stopped and the selected continuation regimen. Remove vinorelbine-related prophylaxis if no longer needed.
- Continue cardiac monitoring; review symptoms and the active-regimen blood schedule.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00798 v3 · p4 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-21 · SACTCheck v0.84.16 · Regimen-level drafts; prescriber and oncology-pharmacy verification required